NCCP 00660 · clinician draft

Fluorouracil + folinic acid every 14 days

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Fluorouracil treatment day

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before the fluorouracil-only treatment visit NCCP NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative.
Metoclopramide	10 mg orally up to three times daily as required	Rescue; prescriber confirms duration Check renal function, neurological contraindications and interactions.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.
- Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00660 v2b · NCCP NCIS antiemetics V8

Sources checked 2026-11-04 · SACTCheck v0.84.32 · Regimen-level drafts; prescriber and oncology-pharmacy verification required